TRAUMA IN PREGNANCY: EVALUATE, MONITOR, DECIDE

FD-ANT-TRAUMA · Companion to Trauma in pregnancy · SOGC CPG No. 325 · EAST 2010 · ACOG · Labor and delivery / OBED · Decision support only, not unit policy

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Last reviewed: Provisional, Nov 2026
PROVISIONAL

HOW TO READ THIS CHART: start at node 1. Hexagons are questions; shaded rectangles are actions. Nodes 5 and 6 apply at **every branch**. **Doses live on the companion card; this chart lists none.** The 4–6 hour timer is a cue only, not a documented time.

SHADING KEY: WHITE = discharge / proceed · LIGHT GRAY = GA branch / monitoring · MID GRAY = extended monitoring · DARK GRAY = unstable mother — stop here

1. IS THE MOTHER UNSTABLE?

Red flags: SBP <90, HR >120, altered mental status, high-energy mechanism, penetrating injury.

YES

UNSTABLE — STOP HERE · DARK GRAY
Trauma team or ED primary, OB at the bedside. Left uterine displacement if 20 weeks or more. Labs plus type and crossmatch. EFM only once the mother is stabilized. **Stop here.**

NO

2. GESTATIONAL AGE?

Choose the branch that matches her gestational age. Viability cutoff: **per local policy**.

UNDER 20 WEEKS · LIGHT GRAY
Doppler FHT. Maternal evaluation. The Rh step (node 5). Then disposition by maternal findings.

20 WEEKS TO UNDER THE LOCAL VIABILITY CUTOFF · LIGHT GRAY
FHT and ultrasound. Rh step. Maternal disposition. Abruption precautions. Viability cutoff: **per local policy**.

VIABLE · WHITE
Go to node 3.

Still complete nodes 5 and 6.

3. EFM AND TOCO · LIGHT GRAY
EFM and toco for at least 4–6 hours from the event. Use the start-time and countdown cue with this algorithm (cue only, not a documented time; nothing is stored).

4. ANY EXTENDED-MONITORING CRITERION?

Contractions 1 or more per 10 minutes, bleeding, uterine tenderness, ROM, a category II or III tracing, or a serious maternal mechanism or injury.

YES

EXTEND MONITORING · MID GRAY
Extend to 24 hours. Add coags and fibrinogen. If abruption or preterm labor is suspected, see ANT-02 or the hemorrhage cards. Steroids and Mg per gestational age (doses on the cards).

NO

DISCHARGE · WHITE
The 4–6 hours were reassuring. Discharge with abruption precautions. Still complete nodes 5 and 6.

5 · RH STEP (EVERY BRANCH)	**Rh-negative:** KB, then RhIG within 72 hours, with extra vials per the KB (dose on the card). **Rh-positive:** no RhIG.
6 · REQUIRED, ANY BRANCH	Private IPV screen. Ultrasound for previa before any digital exam. A normal ultrasound does not rule out abruption.
OBED WALK-IN	Walk-in after minor trauma (not via ED/trauma): send to ED / activate trauma for any red flag, head strike with LOC, suspected fracture, injury beyond the uterus, or care OBED cannot provide. Delayed (>24 h): still evaluate fully. Any contractions, bleeding, pain, leaking or decreased FM → extended monitoring. RhIG within 72 h; beyond 72 h still give and send KB (dose on the card). **Pending house decision** on RhIG >72 h and on observation time after late trauma.
MECHANISM / IPV / FOLLOW-UP	Document mechanism in her words. Strangulation (any neck compression) is a high-lethality marker → ED/forensic evaluation. IPV screen alone; if positive, National Domestic Violence Hotline 1-800-799-7233, text START to 88788, discreetly, not on a printed AVS. Prenatal follow-up within 1 week. Seat-belt and fall-prevention counseling; acetaminophen (avoid NSAIDs ≥20 wk unless directed).

PROVISIONAL — pending Michael review and CSM pharmacy check. Not live. Doses live only on the companion card (Trauma in pregnancy). Decision support only, not unit policy. The 4–6 hour timer is a cue only, not a documented time; nothing is stored.